

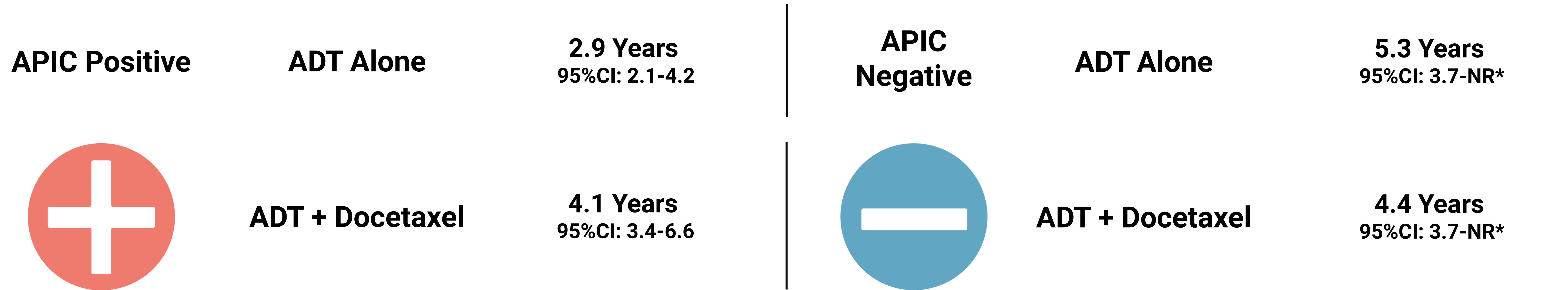
APIC Prostate Test Report

PATIENT	CLINICAL AND PATHOLOGY	ORDERING PHYSICIAN
Name: Date of Birth: --/--/---- ID: Date of Biopsy: --/--/---- Biopsy WSI ID:	Clinical Stage: PSA (ng/mL): Gleason Score: Metastasis Volume: Metastasis Timing: NCCN Risk Group:	Name: Clinic:

PROGNOSTIC SUPPLEMENTAL INFORMATION

In retrospective analysis of 208 metastatic hormone-sensitive prostate cancer patients enrolled in the CHAARTED trial (ECOG-ACRIN E3805) that evaluated the addition of docetaxel to standard androgen deprivation therapy¹:

- **56% (118 patients) were classified as APIC-Positive.** These patients had a significant survival benefit from adding docetaxel to hormone therapy. The 5-year survival rate improved from 16% with hormone therapy alone to 40% with the addition of docetaxel - an absolute difference of 24%. They also experienced a significant delay in developing castration resistance, with median time increasing from 0.7 years to 1.3 years.
- **44% (90 patients) were classified as APIC-Negative.** These patients showed no survival benefit from adding docetaxel to hormone therapy. In fact, their 5-year survival rate was slightly worse with docetaxel (41% vs 56% with hormone therapy alone). They also showed no improvement in time to castration resistance, suggesting they could avoid chemotherapy without compromising outcomes.



APIC v1.0 is a Level 1 automation predictive biomarker classifier validated on H&E prostate core needle biopsies scanned as Aperio SVS format at 20x magnification (0.4598 μm/pixel). Clinical validation performed on CHAARTED (n=208) and NRG/RTOG 0521 (n=266) cohorts demonstrating significant treatment-biomarker interactions (p<0.05). Requires HistoQC artifact removal with pathologist review, and adequate staining. APIC outputs a continuous score. Known limitations include unknown performance on post-treatment biopsies and slides with insufficient tissue. Revalidation required for scanner, staining, or software changes. Research use only, not FDA approved.

1. Medina, Tokuyama, et. al., Computational pathology to predict docetaxel benefit in patients with metastatic hormone-sensitive prostate cancer from the CHAARTED trial (ECOG-ACRIN E3805). *JCO* **43**, 329-329(2025). DOI:10.1200/JCO.2025.43.5_suppl.329
*NR = Not Reached